

Rodney James Foster

9/25/2024 8:45 AM Office Visit

Description: 51 year old male Provider: Norman Hugh Scott McCulloch Jr., MD Department: GSV WOUND CARE OP

Visit Diagnoses

Primary: Diabetic ulcer of other part of left foot associated with type 2 diabetes mellitus, with fat layer exposed (CMS/HCC) E11.621, L97.522
Diabetic polyneuropathy associated with type 2 diabetes mellitus (CMS/HCC) E11.42
History of amputation of great toe (CMS/HCC) Z89.419
Type 2 diabetes mellitus with left diabetic foot infection (CMS/HCC) E11.628, L08.9

Orders Placed

None

Medication Changes

As of 9/25/2024 9:15 AM

None

Medication List at End of Visit

As of 9/25/2024 9:15 AM

	Refills	Start Date	End Date
acarbose (PRECOSE) 100 mg tablet Take 100 mg by mouth 3 (three) times a day with meals. - oral Patient-reported medication	—		—
cephalexin (KEFLEX) 500 mg capsule Take 1 capsule (500 mg total) by mouth 2 (two) times a day for 14 days Indications: an infection of the skin and the tissue below the skin. - oral	0	9/16/2024	9/30/2024
methocarbamol (ROBAXIN) 500 mg tablet Take 1 tablet (500 mg total) by mouth 2 (two) times a day. - oral	0	12/21/2023	—
naproxen (NAPROSYN) 500 mg tablet Take 1 tablet (500 mg total) by mouth 2 (two) times a day with meals. - oral	0	12/21/2023	12/20/2024
pregabalin (LYRICA) 50 mg capsule Take 1 capsule (50 mg total) by mouth 2 (two) times a day. - oral	0	3/23/2023	—
tadalafil (CIALIS) 20 mg tablet TAKE 1 TABLET (20 MG TOTAL) BY MOUTH DAILY AS NEEDED FOR ERECTILE DYSFUNCTION - oral	3	6/5/2024	—
tirzepatide (Mounjaro) 2.5 mg/0.5 mL SQ pen injection Inject 7.5 mg under the skin once a week On Mondays. - subcutaneous Patient-reported medication	—		—

Foster, Rodney James

MRN: 000899552

Norman Hugh Scott McCulloch Jr., MD
Physician
Specialty: Undersea and Hyperbaric Medicine

Progress Notes 
Signed

Encounter Date: 9/25/2024

Provider Wound Follow-up Note**Encounter Diagnoses**

Code	Name	Primary?
E11.621, L97.522	Diabetic ulcer of other part of left foot associated with type 2 diabetes mellitus, with fat layer exposed (CMS/HCC)	Yes
E11.42	Diabetic polyneuropathy associated with type 2 diabetes mellitus (CMS/HCC)	
Z89.419	History of amputation of great toe (CMS/HCC)	
E11.628, L08.9	Type 2 diabetes mellitus with left diabetic foot infection (CMS/HCC)	

Assessment:**Type 2 diabetes mellitus with left diabetic foot infection (CMS/HCC)**

Acute

Recent tissue culture results reviewed and discussed with patient

Heavy growth of E. coli and group B strep

Sensitivities report reviewed

He was prescribed course of oral cephalexin providing coverage

Will continue clinical assessment upon completion of this antibiotic regimen

Diabetic ulcer of other part of left foot associated with type 2 diabetes mellitus, with fat layer exposed (CMS/HCC)

Chronic, complicated

Acute wound infection (refer to DFI plan)

Full thickness ulcer. Pale yellow/red wound bed.

No PTB

Excisional sharp debridement performed down to and including subcutaneous tissue in efforts to remove devitalized tissue, decrease bioburden

C/w 1/4 strength Dakin's solution twice daily

C/w aggressive off-loading

Recent Hgb A1c 7.6%-improved since last value. C/w diabetic medication regimen as prescribed. Ongoing management as per PCP

RTC 2 weeks for NV 10/9

Physician assessment 10/23